



Maternal and child health care

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Out Lines:

- ❑ Introduction.
- ❑ Importance of maternal and child health.
- ❑ The aim of maternal and child health.
- ❑ Objectives of maternal and child health care.
- ❑ Definitions of maternal and child health.
- ❑ MCH centers team.
- ❑ Function of MCH center.

☐ Maternal care :-

A. Family planning

B. Social care.

C. Health education.

D. Program of maternal health:-

1 - Pre-conception care

2- Antenatal care

3- Intra-natal care

4- Postnatal care

☐ Infant and child care.

- ❑ Causes of maternal mortality.
- ❑ Objective of child health services.
- ❑ Services of MCH center for child care.
- ❑ Principles essential for newborn care.
- ❑ Causes of infant mortality.
- ❑ Nursing role in maternal and child health.

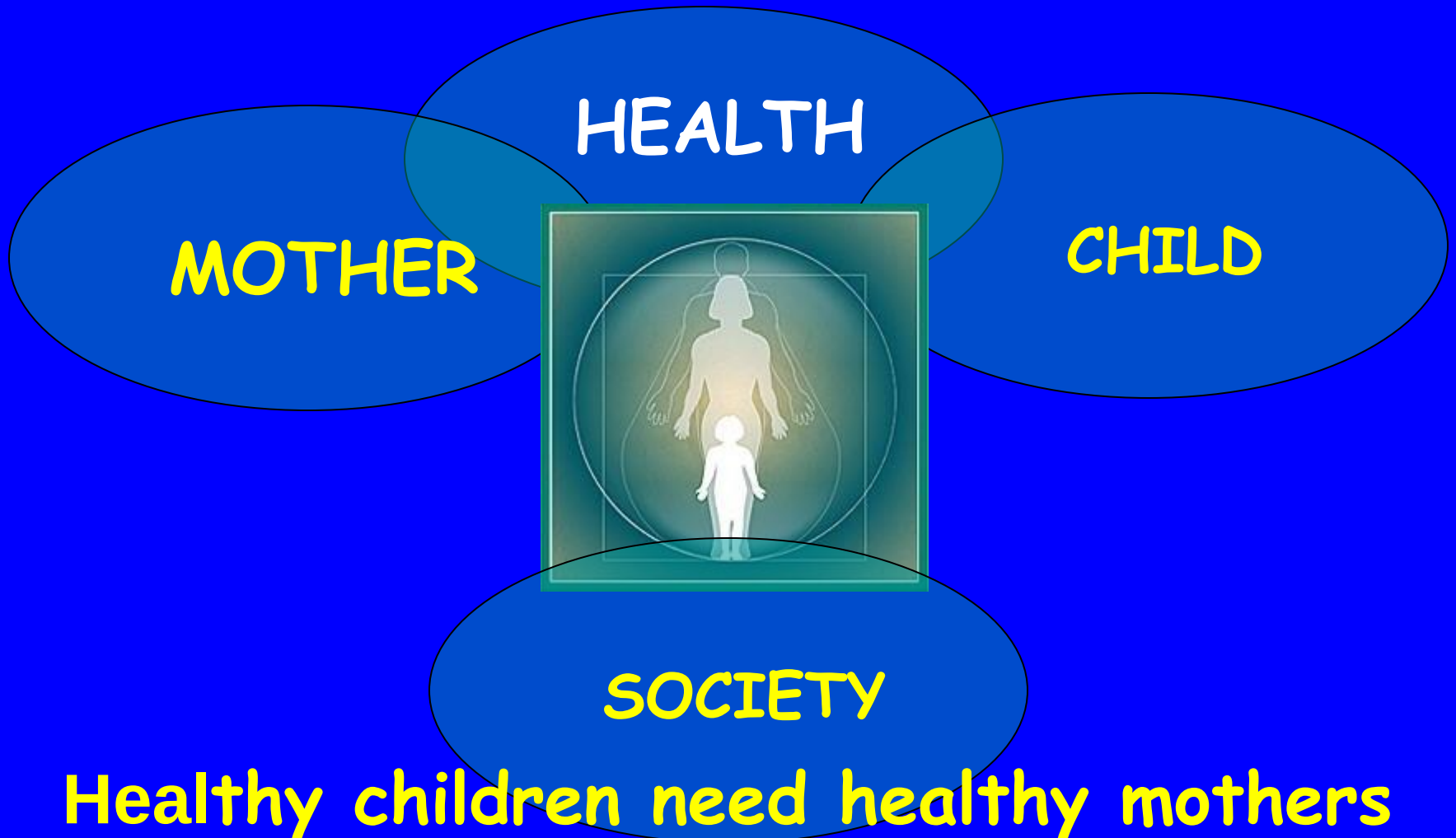
Objectives:

At the end of this lecture the students will be able to:

- Define maternal and child health
- Describe the importance of maternal and child health
- Identify objectives of maternal and child health care.
- Discuss function of MCH center.
- Describe the components of program of maternal health

- Recognize the nursing role in maternal and child health.
- Recognize Causes of maternal mortality.
- Differentiate between direct and indirect causes of maternal mortality.
- Apply health education to women during 1st, 2nd, and 3rd trimester.

Maternal and children health.



Healthy children need healthy mothers

Introduction:-

Mother and children not only constitute a large group, but they are also vulnerable or special group of the community. Women in the childbearing period (15- 49 years) constitute about 25% of the population. Children on the other hand constitute about 40% to 45% of the population in developing countries. This group is characterized by relative high mortality and morbidity rates. The health services of mothers and their children's provided through : MCH center and units of urban and rural areas.

Importance of maternal and child health:

Women and children consist 2/3 of the population, in addition they are the vulnerable segment in the community, as the highest proportion of morbidity as malnutrition, respiratory disease are among this segment . Because the highest proportion of those are with poverty, illiteracy which affect the health status.

The aim of maternal and child health

Improve the health and well-being of women, infants, children, and families.



Objectives of maternal and child health care:

1- Health promotion of mother and child .

2-Prevention of health hazards to the mother during pregnancy, labor, and the puerperium and prevent hazards that facing child.

3- Control of health hazards by health appraisal for early case-finding and proper management of any health problems for mother and child.

Definition maternal and child health :-

Maternal and child health refers to the promoting, preventing and rehabilitating health care of mothers and children up to preschool age. It includes maternal health, child health, and family health etc.

MCH centers team :-

- * Physicians
- * Dentists.
- * Nurses.
- * Pharmacists.
- * Health educators.
- * Clerks.
- * Midwives.
- * Social workers.
- * Laboratory technicians.

Functions of MCH center :-

1. Maternal care :-

A- Family planning

B- Social care.

C- Health education

D- Program of maternal health:-

- Premarital care
- Antenatal care.
- Natal care.
- Postnatal care.

2. Infant and child care.

Maternal care :-

A- Family planning :-

Goal of family planning program:

- ❖ Improve pregnancy planning and spacing.
- ❖ Prevent unintended pregnancy.

Family planning services include:

- ❖ Advice about birth control methods.
- ❖ Birth control pills and devices, such as IUDs.
- ❖ Emergency contraception.
- ❖ Physical exams, including pelvic and breast exam.
- ❖ Pregnancy testing and counseling.
- ❖ Screenings, such as pap smears and sexually transmitted infections when done as part of the family planning visit.
- ❖ Permanent sterilization .

B- Social care

The social worker and the nurse together share the responsibility for prepare the couple for marriage, preparing the family to parenthood role and develop a healthy parent-child relationship through counseling and health education.

C- Health education

- Premarital education.
- Maternity education through antenatal, natal and post natal period.
- Education for mother about infant and preschool child care.

D- Program of maternal health:-

1- Pre-conception care

- It is a care of female before conception.
- It is continued care from birth, through stages of growth and development, and until the time of conception and pregnancy, so as to prepare the female for normal child bearing and delivery in the future.

Purpose of premarital care:

1. Early detection of health problems and early treatment.
2. Providing guidance, preparation for marriage, family education, training in the art of child rearing and family planning.
3. Prevention of health problems for the couple and their future children.
4. Welfare of the new generation where the offspring's can be saved any health hazards of maternal .

Components of premarital care:-

a- Premarital assessment include :-

- Taking complete family and medical history, especially those of genetic or congenital significance.
- Comprehensive physical examination.
- Investigations that include X ray to screen suspected TB cases, blood investigation to diagnoses for syphilis and Rh factor. Urine investigation for albuminuria to investigate kidney disease specially hypertension and glucose urea to diagnose diabetes.

b- Premarital education includes:-

Health education about :-

1. Nutrition.
2. Family planning guidance.
3. Family life education.
4. Child caring education.

c- premarital counseling:-

About personal hygiene, dietary counseling , sexual counseling and successful marriage.

Gather counseling model.

Gather counseling model can be used in premarital counseling it includes six elements:

- * **Greeting the couples:** Introduce yourself, learn their names and inform them that the conversation is confidential.
- * **Asking about them:** Collect relevant information .
- * **Tell them:** Relevant information they may need.
- * **Help them:** To make decisions regarding their future lives.
- * **Explain:** About their choices.
- * **Return:** For follow up visit.

d- premarital immunization :- German measles and mumps.

2- Antenatal care (Prenatal care)

Antenatal care should be start in, and continued through pregnancy.

Goals of antenatal care :

- Health promotion and safety of mother and her fetus.
- Favorable outcome.
- Early recognition and management of high risk pregnancies.

- The preparation of mother for labor, lactation and subsequent care of her child.
- The reduction of maternal and infant mortality, stillbirth, and prematurity.
- To increase the number of breastfeed babies.

B) Examination in antenatal periods:

General examination Includes:

- Weight, height, blood pressure.
- Examination of the eye.
- Examination of the mouth and dental examination.
- Examination of the neck for goiter.
- Examination of heart and chest
- Abdominal examination for detect lenia negra, steria gravidarum or any scares of previous operation and to check skin condition.
- Examination of the lower extremities to detect edema and varicose, veins.

Components of antenatal care:-

- A) Registration:** Initial & return visit.
- B) Examination:** General ,and abdominal.
- C) Health education** about breast feeding and breast care.
- D) Immunization.**
- E) Social service .**



A)Registration: 1-The initial visit (first visit):

Basic data on first visit includes:

Personal history: Name, age, address, and occupation.

Family history : Family history of any disease.

Medical history: History of any operation or disease.

Present history: If the mother have any disease in the present time.

Obstetrical history: As number of gravida , labour, and abortion.

Menstrual history: Which includes, age of menarche, duration, frequency, and last menstrual period to determine the expected date of delivery.

Local examination:

Examination of the breast for:

- Nipple to detect inverted or cracked nipple.
- To show areola.
- Size and tenderness of the breast.
- Detection breast discharge if present.
- Inspection of any mass.

2- Return visit:

The mother is asked to attend the antenatal clinic

- Once every month through the first six months.
- One every two weeks through the next two months.
- And then weekly until the time of delivery.
- She can also seek medical care and clinical services at any time.

C) Health education :-In first trimester, teach the woman about:

- **Diet:** Eat simple , frequent and easily digested diet.
She should be take high protein diet.
- **Morning sickness:** The woman should take dry piece of carbohydrate food a half an hour before getting out of bed.
- **Frequency of micturition:** Teach the women that it is normal in first trimester due to pelvic congestion.

- **Flatulence:** She should eat only small amount of food and masticating it well, regular daily elimination, avoidance of food that form gases.
- **Avoidance of high heels and tight clothing.**
- **Bathing and traveling .**
- **Rest, sleep and exercises.**
- **Pathological manifestation** special toxemias (blurred vision, headache, persistent vomiting, edema and bleeding) to seek medical consultation early.
- **Personal hygiene .** And avoidance of exposure to infectious diseases.

Second trimester:

Second trimester is quite period in pregnancy.

Health education to be given to the mother about:

- Nutrition: Good balanced diet should be taken.
- Rest, activity, exercise, and sleep.
- Pathological manifestation .
- Personal hygiene.

- Avoidance of tight clothing, she should wear broad clothes.
- Principles of child welfare and maternal responsibly as caring for the nipples and preparing for feeding, cleanliness and management of illness.
- Family planning methods and its effectiveness.

Third trimester: Health education about:

- Dyspnea
- Heart burn
- Edema
- Vaginal discharge
- **Also education about:**
 - Cramps
 - Backache
 - Varicose veins
- Preparation of the place of delivery, mother and baby clothes, baby care, immunization, and feeding.
- Signs of delivery and significant pathological manifestations specially toxemias in order to seek medical consultation at once.

D) Immunization against tetanus.

E) Social services:

Cases with social problem are investigated by social worker or health visitor to give the necessary facilities and aids. Foods as dry milk, cheese and flour can be provided to support nutrition.



3- Natal care:

Objectives:

- 1- Safety of mother and fetus.
- 2- Assist the mother to have normal delivery.
- 3- Provide emergency service if needed.

Normal delivery is defined as a process of delivery of a single fetus and other products of conception within 24 hours, through the normal birth canal and without complications.

Place of delivery: At home or in the MCH center or hospital.

4- Postnatal care: It include:-

General examination:

- Examination of general condition of the mother.
- Check vital signs as temperature, pulse, blood pressure, to detect fever and bleeding.

Local examination:

- Examination of the breast to detect any abnormalities as cracked and inverted nipple, and show milk secretion.
- Abdominal examination to check the involution of the uterus at the day of delivery. The fundal level above the umbilicus by one finger and every day, the fundal level decreased one finger.
- Complete involution of the uterus at 15 days after delivery.
- Examination of perineum and vagina to detect any (tears, lacerations, and vaginal bleeding or discharge.

Health education: It includes nutritional education, general cleanliness, principles of child welfare and exercises for abdominal muscles.

Causes of maternal mortality:

1- Medical causes:-

A. Direct obstetric causes:

Pregnancy, labor, postnatal period, incorrect treatment ,toxemia ,hemorrhage ,infection ,obstructive labor ,and unsafe abortion.

B- Indirect obstetric causes:

Resulting from previous existing disease or disease that developed during pregnancy (anemia, associated diseases: Cardiac, renal, metabolic, infection, malignancy and accidents).

2- Social causes:-

Age at child birth, , poverty, illiteracy, ignorance and poor environmental sanitation.

Objective of child health services:

- Health promotion of children.
- Prevention and control of diseases and hazards.
- Prevention of disability and rehabilitation.
- Reducing child mortality.

Services of M.C.H. centers for child care:

1. Preventive services which include:

a. Prenatal and natal care:

- Good antenatal care for normal fetal growth and ensure safe delivery of the fetus.



b . Newborn care:

- Asepsis with cutting umbilical cord and dressing.
- Clean airway passage and restoration of normal respiration.
- Management of any complication if present.
- Cleanliness of the eye, mouth and body.
- **Do Apgare score as follow:**

Apgare score Sign	0	1	2
Heart rate	Absent	Slow (below 100/min)	Over 100/min
Respiratory rate	Apnea	Weak, irregular gasping	Regular, good, strong Cry
Muscle tone	Flaccid (limp)	Some flexion of extremities	Good flexion of arms and legs
Reflex irritability	Not response	Some response	Facial grimace cough, sneeze
Color	Blue, pale	Body pink and extremities blue	Completely pink

- Apgare score should be done in 1 minute and 5 minute after delivery.
- The scoring is score of 7 - 10 normal.
- 4- 6 infant require resuscitation.
- from 0 - 3 severely depressed infant.

c-Health promotion of infants and preschool children:

- Encourage of breast feeding.
- Supplementation of milk for infants and twins.
- Nutritional education about how to prepare adequate diet, proper breast feeding and weaning practices.

d- Prevention and control of communicable diseases through:-

- Immunization of infants and children.
- Health education about personal hygiene, and food.
- Environmental sanitation and prevention of infection.

e- Maintenance of health through periodic medical examination:

- Periodic examination of the child for checkup of the general condition and growth and development through regular visits to MCH .
- Every child should have own health record, which contains information from birth till the end of preschool period.

2- Curative services:

MCH centers provide curative services for sick children through:

- Case finding .
- Early diagnosis, treatment and follow up of cases.
- Referral of cases.

Causes of Infant Mortality

Neonatal mortality

- Low birth weight (LBW) and prematurity.
- Birth injury and difficult labor.
- Sepsis.
- Congenital anomalies.
- Hemolytic disease of newborn.
- Conditions of placenta and cord.
- Diarrhea, Acute respiratory infection and tetanus.

Post neonatal mortality

- Diarrhea.
- Acute respiratory infection(ARI).
- Malnutrition.
- Congenital anomalies.
- Accidents.

For developing appropriate MCH services:

- 1 .Adaptation to ecology and needs.
- 2 .Real community participation .
3. Economic constraints.
4. Integration within general health services.
- 5.“At – risk” in focus.

6. Education.

7. Trained staff.

8. Guided evaluation.

9. Acceptable within national development planning activities.

10. Re-assess priorities.

11. Consider cost- effectiveness.

Nursing role in maternal and child health care:-

- 1- Innovation in the delivery of early year's services means nurses increasingly work alongside other professionals to deliver integrated care for child and mother.
- 2- Nurse role are support, information, health promotion, the early identification of health concerns and appropriate intervention.
- 3- Nurse help in marriage counseling and sex education and encourage periodic examination .

4- Nurses are supported through ongoing training and education opportunities to deliver integrated care for child and mother.

5- The nurse support and advice regarding child health, development, child behavior, maternal health and wellbeing, child safety, immunization, breastfeeding, nutrition and family planning.

6- The nurse uses educational interventions when teaching about family planning, nutrition, safety precautions, and child care skills.

7- Intervention include strategies in which the nurse uses a positive manipulation, such as conducting voluntary immunization programs, and encouraging smoking cessation during pregnancy.

8- The nurse provide information to young mothers about infant sleep schedules, colic, toilet training, nutrition and feeding.

9- The nurses work with all families with children from birth to school age to support them.

10- Cooperates with other health workers in the health team.

11- MCH nurse provide early detection of high risk group and treatment of sick children.

12- Referral and links to specialist services and maintain follow up for mother and child.

13- Participates in the organization of the MCH program.

THANK YOU

